

360 mm

PHARMACODE READING
DIRECTION

Front

110 mm

Back

110 mm



Travo Eye Drops

Travoprost 0.004% w/v Eye Drops, Solution

NAME AND STRENGTH OF ACTIVE INGREDIENT:

Each ml contains:
Travoprost USP 0.004% w/v
Benzalkonium Chloride Ph. Eur. 0.015% w/v
(As preservative)
Water for Injections Ph. Eur. q.s. to 100% w/v

PRODUCT DESCRIPTION: Clear, colorless to pale yellow solution practically free from particles.

Pharmacodynamics:

Travoprost, a prostaglandin $F_{2\alpha}$ analogue, is a highly selective full agonist which has a high affinity for the prostaglandin FP receptor, and reduces the intraocular pressure by increasing the outflow of aqueous humour via trabecular meshwork and uveoscleral pathways. Reduction of the intraocular pressure in man starts about 2 hours after administration and maximum effect is reached after 12 hours. Significant lowering of intraocular pressure can be maintained for periods exceeding 24 hours with a single dose.

Pharmacokinetics

Absorption: Travoprost is an ester prodrug. It is absorbed through the cornea where the isopropyl ester is hydrolyzed to the active free acid. Studies in rabbits have shown peak concentrations of 20 ng/mL of the free acid in aqueous humour one to two hours after topical dosing of Travoprost. Aqueous humour concentrations declined with a half-life of approximately 1.5 hours.

Distribution: Following topical ocular administration of Travoprost to healthy volunteers, low systemic exposure to active free acid was demonstrated. Peak active free acid plasma concentrations of 25 pg/mL or less were observed between 10 and 30 minutes post-dose. Thereafter, plasma levels declined rapidly to below the 10 pg/mL assay quantitation limit before 1 hour post-administration. Due to the low plasma concentrations and rapid elimination following topical dosing, the elimination half-life of active free acid in man could not be determined.

Biotransformation: Metabolism is the major route of elimination of both Travoprost and the active free acid. The systemic metabolic pathways parallel those of endogenous prostaglandin $F_{2\alpha}$, which are characterized by reduction of the 13-14 double bond, oxidation of the 15-hydroxyl and β -oxidative cleavages of the upper side chain.

Elimination: Travoprost free acid and its metabolites are mainly excreted by the kidneys. Travoprost has been studied in patients with mild to severe hepatic impairment and in patients with mild to severe renal impairment (creatinine clearance as low as 14 mL/min). No dosage adjustment is necessary in these patients.

INDICATIONS: Decrease of elevated intraocular pressure in adult patients with ocular hypertension or open-angle glaucoma.

RECOMMENDED DOSE:

Use in adults, including the elderly population
The dose is one drop of Travoprost eye drops in the conjunctival sac of the affected eye(s) once daily. Optimal effect is obtained if the dose is administered in the evening.
Nasolacrimal occlusion or gently closing the eyelid after administration is recommended. This may reduce the systemic absorption of medicinal products administered via the ocular route and result in a decrease in systemic adverse reactions.
If more than one topical ophthalmic medicinal product is being used, the medicinal products must be administered at least 5 minutes apart.
If a dose is missed, treatment should be continued with the next dose as planned. The dose should not exceed one drop in the affected eye(s) daily.
When substituting another ophthalmic Antiglaucoma agent with Travoprost eye drops, the other agent should be discontinued and Travoprost eye drops should be started the following day.
Paediatric population
The efficacy and safety of Travoprost eye drops in patients below the age of 18 years have not been established and its use is not recommended in these patients until further data become available.
Hepatic and renal impairment Travoprost eye drops has been studied in patients with mild to severe hepatic impairment and in patients with mild to severe renal impairment (creatinine clearance as low as 14 mL/min). No dosage adjustment is necessary in these patients.

MODE OF ADMINISTRATION: For ocular use

The patient should remove the protective overwrap immediately prior to initial use. To prevent contamination of the dropper tip and solution, care must be taken not to touch the eyelids, surrounding areas or other surfaces with the dropper tip of the bottle.

CONTRAINDICATIONS: Hypersensitivity to the active substance or to any of the excipients

WARNINGS AND PRECAUTIONS:

Eye color change

Travoprost may gradually change the eye color by increasing the number of melanosomes (pigment granules) in melanocytes. Before treatment is instituted, patients must be informed of the possibility of a permanent change in eye color. Unilateral treatment can result in permanent heterochromia. The long term effects on the melanocytes and any consequences thereof are currently unknown. The change in iris color occurs slowly and may not be noticeable for months to years. The change in eye color has predominantly been seen in patients with mixed colored irides, i.e., blue-brown, grey-brown, yellow-brown and green-brown; however, it has also been observed in patients with brown eyes. Typically, the brown pigmentation around the pupil spreads concentrically towards the periphery in affected eyes, but the entire iris or parts of it may become more brownish. After discontinuation of therapy, no further increase in brown iris pigment has been observed.

Periorbital and eye lid changes

Periorbital and/or eyelid skin darkening in association with the use of Travoprost has been reported in patients. Periorbital and lid changes including deepening of the eyelid sulcus have also been observed with prostaglandin analogues.

Travoprost may gradually change eyelashes in the treated eye(s); these changes were observed in about half of the patients and include: increased length, thickness, pigmentation, and/or number of lashes. The mechanism of eyelash changes and their long term consequences are currently unknown.

Travoprost has been shown to cause slight enlargement of the palpebral fissure in studies in the monkey. However, this effect was not observed and is considered to be species specific. There is no experience of Travoprost in inflammatory ocular conditions; nor in Neovascular, angle-closure, narrow-angle or congenital glaucoma and only limited experience in thyroid eye disease, in open-angle glaucoma of pseudophakic patients and in pigmentary or pseudoexfoliative glaucoma. Travoprost should therefore be used with caution in patients with active intraocular inflammation.

Aphakic patients: Macular oedema has been reported during treatment with prostaglandin $F_{2\alpha}$ analogues. Caution is recommended when using Travoprost in aphakic patients, pseudophakic patients with a torn posterior lens capsule or anterior chamber lenses, or in patients with known risk factors for cystoid macular oedema.

Iritis/uveitis: In patients with known predisposing risk factors for iritis/uveitis, Travoprost should be used with caution.

Contact with the skin

Skin contact with Travoprost must be avoided as transdermal absorption of Travoprost has been demonstrated in rabbits. Prostaglandins and prostaglandin analogues are biologically active materials that may be absorbed through the skin. Women who are pregnant or attempting to become pregnant should exercise appropriate precautions to avoid direct exposure to the contents of the bottle. In the unlikely event of coming in contact with a substantial portion of the contents of the bottle, thoroughly cleanse the exposed area immediately.

Contact lenses: Patients must be instructed to remove contact lenses prior to application of Travoprost and wait 15 minutes after instillation of the dose before reinsertion.

Excipients

Travoprost contains propylene glycol which may cause skin irritation.

Travoprost contains polyoxyethylene hydrogenated castor oil 40 which may cause skin reactions.

Paediatric population: Efficacy and safety data in the age group 2 months to < 3 years (9 patients) is limited. No data are available for children below the age of 2 months.

In children < 3 years old that mainly suffer from PCG (primary congenital glaucoma), surgery (e.g. trabeculotomy/goniotomy) remains the first line treatment. No long-term safety data are available in the paediatric population.

Effect on ability to drive and use machines: Travoprost has no or negligible influence on the ability to drive and use machines, however as with any eye drop, temporary blurred vision or other visual disturbances may affect the ability to drive or use machines. If blurred vision occurs at instillation, the patient must wait until the vision clears before driving or using machines.

INTERACTIONS WITH OTHER MEDICAMENTS: No interaction studies have been performed.

PREGNANCY AND LACTATION:

Women of child-bearing potential/contraception

Travoprost eye drops must not be used in women of child bearing age/potential unless adequate contraceptive measures are in place.

Pregnancy: Travoprost has harmful pharmacological effects on pregnancy and/or the foetus/new-born child. Travoprost eye drops should not be used during pregnancy unless clearly necessary.

Breast feeding: It is unknown whether Travoprost from eye drops is excreted in human breast milk. Animal studies have shown excretion of Travoprost and metabolites in breast milk. The use of Travoprost eye drops by breast-feeding mothers is not recommended.

UNDESIRABLE EFFECTS:

Travoprost, the most common adverse reactions were ocular hyperaemia and iris hyperpigmentation, occurring in patients respectively.

Tabulated list of adverse reactions

System Organ Class	Frequency	Adverse Reactions
Infections and infestations	Rare	herpes simplex, keratitis herpetic
Immune system disorders	Uncommon	hypersensitivity, seasonal allergy
Psychiatric disorders	Not known	depression, anxiety
Nervous system disorder	Uncommon	headache, dizziness, visual field defect
	Rare	dysgeusia,
	Very common	ocular hyperaemia,
	Common	iris hyperpigmentation, eye pain, ocular discomfort, dry eye, eye pruritus, eye irritation
	Uncommon	corneal erosion, uveitis, iritis, anterior chamber inflammation, keratitis, punctate keratitis, photophobia, eye discharge, blepharitis, erythema of eyelid, Periorbital oedema, eyelids pruritus, visual acuity reduced, vision blurred, lacrimation increased, conjunctivitis, ectropioncataract, eyelid margin crusting, growth of eyelashes, eyelash discoloration, asthenopia
	Rare	Iridocyclitis, eye inflammation, photopsia, eczema eyelids, conjunctival oedema, halo vision, conjunctival follicles, hypoaesthesia eye, meibomianitis, anterior chamber pigmentation, mydriasis, eyelash thickening
Ear and labyrinth disorders	Not known	macular oedema, sunken eyes
	Not known	vertigo, tinnitus
Cardiac disorders	Uncommon	palpitations,
	Rare	heart rate irregular, heart rate decreased
	Not known	chest pain, bradycardia, tachycardia
Vascular disorders	Rare	blood pressure diastolic decreased, blood pressure systolic increased, hypotension, hypertension
Respiratory, thoracic and mediastinal disorders	Uncommon	dyspnoea, asthma, nasal congestion, throat irritation
	Rare	respiratory disorder, oropharyngeal pain, cough, dysphonia
	Not known	asthma aggravated
Gastrointestinal disorders	Rare	peptic ulcer reactivated, gastrointestinal disorder, constipation, dry mouth
	Not known	diarrhoea, abdominal pain, nausea
Skin and subcutaneous tissue disorders	Uncommon	skin hyperpigmentation (Periocular), skin discoloration, hair texture abnormal, hypertrichosis
	Rare	dermatitis allergic, dermatitis contact, erythema, rash, hair colour changes, madarosis
	Not known	pruritus, hair growth abnormal
Musculoskeletal and connective tissue disorders	Rare	musculoskeletal pain
	Not known	arthralgia
Renal and urinary disorders	Not known	dysuria, urinary incontinence
General disorders and administration site conditions	Rare	asthenia
Investigations	Not known	prostatic specific antigen increased

OVERDOSAGE AND TREATMENT:

No cases of overdose have been reported. A topical overdose is not likely to occur or to be associated with toxicity. A topical overdose of travoprost may be flushed from the eye(s) with lukewarm water. Treatment of a suspected oral ingestion is symptomatic and supportive.

STORAGE CONDITION:

Store below 30°C. Keep medicine out of reach of children.
Discard 28 days after first opening.

DOSAGE FORMS AND PACKAGING AVAILABLE:

Dosage form: Eye drops.
Packaging: 2.5 mL of solution is filled in BFS (Blow Fill Seal) Container.

SHELF LIFE:

24 months from the date of manufacture.
Discard 28 days after first opening.

Date of Revision of Package Insert:

December 2019

Manufactured by:

MICRO LABS LIMITED
No. 113 to 116, 4th Phase, K.I.A.D.B.,
Bommasandra Industrial Area,
Bangalore-560 099, INDIA.

EXG-ML14I-0111/A



Size: 110 (L) x 360 (H) mm

(Final folding size :110x25±1x1.25mm)

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